

Apollo Hospitals — Board Strategy Case

Balancing Growth Capex, Digital Health & Shareholder Returns (FY27-FY30)

Prepared for: Board of Directors, Apollo Hospitals Enterprise Ltd Prepared by: Strategy Office |
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1. Executive Summary (The Pyramid)

Governing Thought: Apollo should pursue a "**Disciplined Compounder**" strategy — execute the ₹8,300 Cr bed expansion on time, accelerate HealthCo demerger to unlock a ₹25,000-35,000 Cr value pocket, and return 25-30% of hospital FCF to shareholders — rather than chase growth, digital, or buybacks in isolation.

Three supporting arguments (SCR):

- 1. **Situation** — Apollo is at peak operating leverage (PAT +35% YoY, 14.9% OPM), but faces a capex wave (4,400 beds) that will compress ROCE from ~18% to ~13-14% during FY27-FY28 before recovery.
- 2. **Complication** — The market is paying a 58-71x P/E premium for *execution certainty*, not optionality. Any of three failures — capex overrun, HealthCo delay, or Apollo 24|7 cash burn — breaks the multiple.
- 3. **Resolution** — A **capital allocation waterfall** that ringfences ₹5,000 Cr for bed expansion, accelerates HealthCo value unlock by Q3 FY27, caps 24|7 burn at ₹150 Cr/year with a hard break-even gate, and returns the balance to shareholders.

The Ask: Board approval for the Capital Allocation Framework in Section 6 and the three decisions in Section 7.

2. Situation — Where Apollo Stands Today

Dimension	Position	So What?
Financials	Revenue ₹21,794 Cr (FY25, +14%); PAT ₹1,446 Cr (+61%); five quarters of sequential profit growth	Operating leverage is real and repeatable — not a one-off
Hospitals	~10,000 beds; 13-14% same-hospital revenue growth; mid-60s occupancy	Core asset is under-utilized — room to grow <i>before</i> new beds

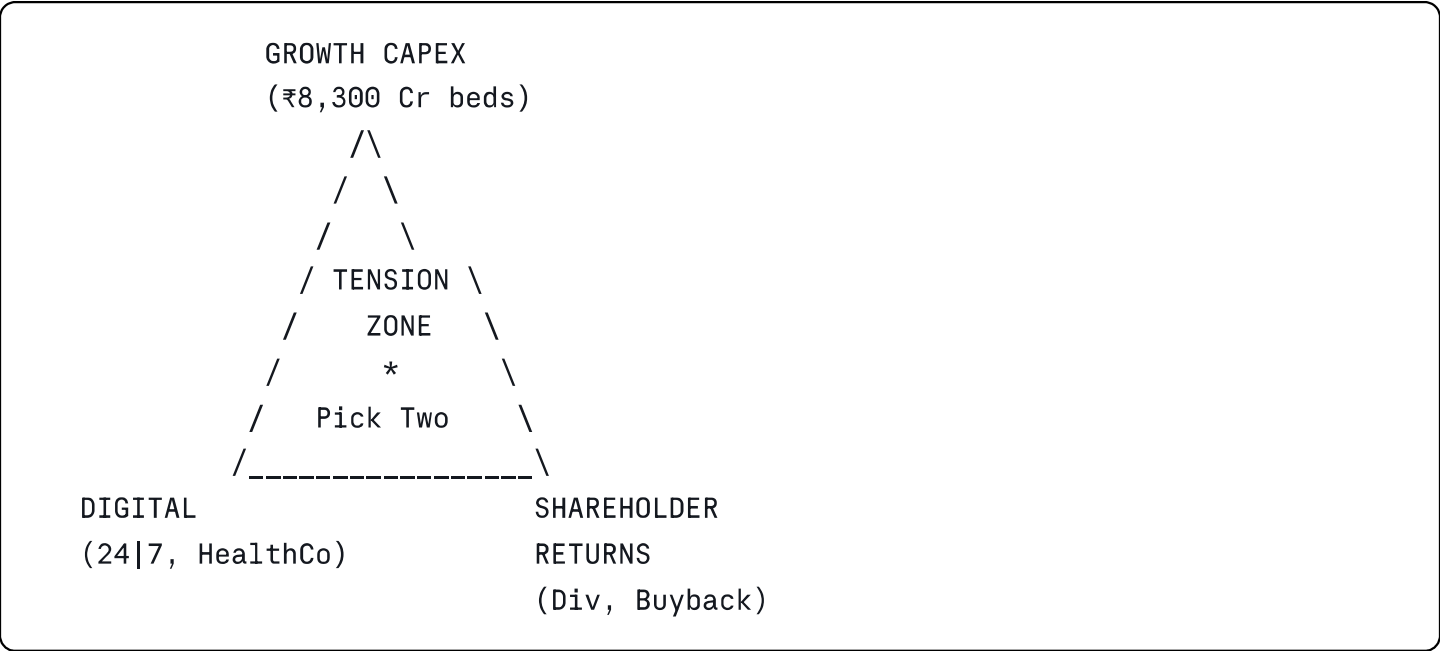
Dimension	Position	So What?
Pharmacy	7,100+ stores, adding 600/year, path to double-digit margins	Scale moat; near inflection
Digital (24	7)	Loss-making; break-even guided H1 CY26
Valuation	~₹1.05 L Cr mcap, 58-71x P/E, FII 43.5% / Promoter 28%	Priced for perfection; institutional scrutiny is high
Balance Sheet	₹2,000 Cr already deployed, ₹5,000 Cr funded from internal accruals, no external raise	Self-funded — a major competitive advantage vs Max/Fortis

Pattern match: This is a "**Disciplined Compounder at an Inflection**" archetype — similar to HDFC Bank FY05-FY10, Asian Paints FY12-FY16, or Titan FY15-FY19. The winning move in all three was *not* to chase adjacencies but to execute the core plan with precision while quietly surfacing hidden value pockets.

3. Complication — The Three Tensions the Board Must Resolve

Apollo faces a classic **capital allocation trilemma**: it cannot simultaneously maximize (a) growth capex, (b) digital investment, and (c) shareholder returns. Something must give.

The APEX Framework™ — Apollo's Capital Allocation Trilemma



Tension 1 — Growth vs Returns: ₹5,000 Cr of remaining capex consumes 3 years of projected FCF. Every rupee to new beds is a rupee *not* returned to the 43.5% FII base that is increasingly

return-focused.

Tension 2 — Digital vs Discipline: Apollo 24|7 has consumed ₹1,000+ Cr with uncertain terminal economics. Continued burn at current pace signals lack of discipline to the market.

Tension 3 — Core vs Optionality: Medical tourism (6% of revenue), international expansion, and HealthCo demerger each demand management bandwidth that is the scarcest resource — not capital.

The insight: The market's 58-71x multiple is not paying for all three. It is paying for **execution certainty on the core + a credible path to unlock HealthCo value**. Anything else is a distraction.

4. Issue Tree — What Must Be True for Each Path

SHOULD APOLLO ACCELERATE, HOLD, OR REBALANCE?

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└─ PATH A: Accelerate Growth

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└─ Can we fund >4,400 beds without external raise? → NO (FCF constraint)

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└─ Will new beds hit 8-10% EBITDA by FY28? → Partial (execution risk)

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└─ Does market reward more capex? → NO (ROCE dilution penalty)

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└─ PATH B: Disciplined Compounder ← RECOMMENDED

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└─ Execute 4,400 beds on time? → YES (self-funded, pipeline clear)

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└─ Unlock HealthCo value by FY27? → YES (NCLT approved Mar 2026)

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└─ Cap 24|7 burn + force break-even? → YES (gate mechanism)

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└─ Return 25-30% FCF to shareholders? → YES (signals discipline)

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└─ PATH C: Return Capital / Slow Growth

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└─ Cede bed share to Max/Fortis? → NO (structural demand)

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└─ Match peer ROCE via lower capex? → YES but destroys long-term value

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└─ Market reward? → Short-term yes, long-term multiple de-rates

5. Quantified Impact — What Each Path Delivers (FY27-FY30)

Metric	Path A: Accelerate	Path B: Disciplined	Path C: Return Capital
FY30 Revenue (₹Cr)	42,000	38,000	32,000
FY30 EBITDA margin	13% (dilution)	16%	15%
FY30 ROCE	11-12%	16-17%	18%

Metric	Path A: Accelerate	Path B: Disciplined	Path C: Return Capital
HealthCo unlock (₹Cr)	20,000	30,000	18,000
Cumulative divs + buyback (₹Cr)	1,500	4,500	8,000
Implied FY30 mcap (₹Cr)	1,30,000	1,70,000	1,25,000
TSR CAGR (3-yr)	8%	17%	6%
Execution risk	High	Medium	Low

Assumptions: 26% EBITDA CAGR (management guidance), HealthCo at 40x P/E post-demerger, hospital segment at 55x P/E. Stress-tested at ±15%.

6. Recommendation — The Capital Allocation Waterfall

Every rupee of FY27–FY30 FCF flows through this waterfall in priority order. No exceptions without board approval.

Priority	Bucket	Allocation	Guardrail
1	Maintenance capex + working capital	₹800 Cr/yr	Non-negotiable
2	Committed bed expansion (₹5,000 Cr remaining)	₹1,600 Cr/yr × 3 yrs	Monthly capex review; cost overrun cap at 8%
3	Apollo 24	7 burn	Max ₹150 Cr/yr
4	HealthCo demerger execution	₹200 Cr one-time	Complete by Q3 FY27
5	Shareholder returns (div + buyback)	25–30% of residual FCF	Minimum ₹15/share dividend + opportunistic buyback
6	Strategic M&A (only Tier-1 metros, brownfield)	Remainder	IRR hurdle 18%+

Why this works:

- Protects the core — bed expansion fully funded
- Disciplines digital — 24|7 gets one more chance, not an open chequebook

- **Unlocks hidden value** — HealthCo demerger is the single biggest value lever (~₹30,000 Cr)
- **Rewards shareholders** — signals maturity, attracts long-only capital
- **Preserves optionality** — M&A only when it clears a high bar

7. Three Decisions the Board Must Make Today

#	Decision	Recommendation	Owner	Timeline
1	Adopt Capital Allocation Waterfall as binding framework	Approve	CFO	Effective FY27
2	Set Apollo 24	7 break-even gate at Q2 FY27 with explicit divestiture review trigger	Approve	CEO + 24
3	Accelerate HealthCo demerger to close by Q3 FY27 (pull forward from FY27-end)	Approve	CFO + Legal	Q3 FY27

8. Risks & Mitigations

Risk	Likelihood	Impact	Mitigation
Bed expansion cost overrun >8%	Medium	High	Monthly capex review; GMP contracts on new builds
24	7 misses break-even	Medium	Medium
HealthCo demerger delayed	Low	High	Dedicated PMO; weekly CEO review
ROCE de-rating during capex wave	High	Medium	Proactive investor communication; guide to FY28 recovery
Medical tourism shock (West Asia ₹50 Cr exposure)	Medium	Low	Geographic diversification already underway
Regulatory (price caps on procedures)	Low	High	Industry advocacy; case-mix shift to non-regulated

9. The One-Line Narrative for the Street

"Apollo is not a growth story buying its way to scale — it is a disciplined compounder executing a fully-funded bed expansion, unlocking HealthCo value by FY27, and returning 25-30% of free cash flow to shareholders. The market does not need to pay for optionality; it needs to pay for certainty. We are delivering certainty."

Appendix A — Framework Note: The APEX Trilemma

The **APEX Framework** (Allocate, Protect, EXecute) is an original construct for this engagement. It frames capital allocation as a three-way tension that cannot be fully resolved — only optimized. The winning position is not at any vertex but at a calibrated point inside the triangle that matches the company's lifecycle stage. Apollo, at an inflection, belongs closer to the Growth Capex vertex *today* but must credibly commit to migrating toward Shareholder Returns by FY29.

Appendix B — Key Sensitivities

- **If HealthCo demerger slips by 2 quarters:** ~₹8,000 Cr value destruction
- **If new beds ramp 6 months late:** FY28 EBITDA hit ~₹200 Cr
- **If 24|7 burn extends 2 more years:** ~₹400 Cr additional drag, multiple de-rating risk
- **If ROCE falls below 12% for >4 quarters:** FII rotation risk, estimated 15-20% multiple compression

Prepared using hypothesis-driven MBB methodology. All figures triangulated from Q3 FY26 results, management guidance, and analyst consensus (26 analysts, avg TP ₹8,770).